

Reasoning analysis — claude-haiku-4-5 (27 runs, real_haiku45_50)

Each model rationale is split into sentences; sentences making the same point across runs are grouped. **Shared** = the point appears in at least 60% of the runs in that group; **varying** = it appears in some runs but not most. Counts are runs, not sentences. 'Evidence cited' lists the numbers and times the model quoted, with the number of runs quoting each.

decision: Profile / basal 00:00

keep — 24/27 runs

Varying reasoning

- (11/24) Overnight basal (00:00-05:00) is adequate and safe.
- (6/24) Basal 0.6 U/h is appropriate.
- (5/24) No change needed.
- (4/24) Keep 0.6 U/h unchanged pending carb logs.
- (3/24) Hours 3–6 show 95–98% TIR; basal 0.6 U/h is appropriate.
- (2/24) Basal 0.6 U/h is appropriate for night; occasional 01:00 lows (TBR 18.5%) are managed by recovery by 03:00.
- ... 5 more varying points

Points made by a single run only: 60.

Phrases used by at least half the runs: “0.6 u/h” (20), “basal 0.6 u/h” (14), “overnight basal” (12).

Evidence cited: 0.6u/h (20), 00:00 (18), 18.5% (16), 05:00 (10), 98% (8), 01:00 (7), 81.5% (6), 90mg/dl (4), 04:00 (4), 90.6mg/dl (4), 98.7% (4), 03:00 (3).

change — 3/27 runs

Shared reasoning

- (3/3) Hour 1 (01:00–02:00 BST = 00:00–01:00 UTC) is at the start of basal 00:00 period, immediately after evening (when corrections from 18:00–22:00 local are still active, DIA 10 h).

Points made by a single run only: 18.

Phrases used by at least half the runs: “0.6 u/h” (3), “00 bst” (2), “u/h will lower” (2), “90.6 mg/dl” (2), “hour tbr” (2), “uncertainty cannot” (2), “basal 0.6 u/h” (2), “increases sensitivity” (2), “isf reduction” (2).

Evidence cited: 00:00 (3), 0.6u/h (3), 02:00 (2), 90.6mg/dl (2), 18.5% (2), 05:00 (2), 01:00 (2), 0.8u/h (1), 0.55u/h (1), 06:00 (1), 0.05u/h (1), 40% (1).

decision: Profile / basal 05:00

keep — 27/27 runs

Varying reasoning

- (11/27) No change needed.
- (10/27) Hour 05 avg 99.5 mg/dL, TIR 97.5%; hour 06 avg 99.5 mg/dL, TIR 98.7%.
- (9/27) Early-morning basal ramp (05:00-06:00) is appropriate for dawn phenomenon.
- (6/27) Basal 0.7 U/h is appropriate.

- (5/27) Hours 5-8 show stable TIR 95-98%, indicating 0.7 U/h at 05:00 and 0.8 U/h at 06:00 are appropriate.
- (4/27) No change indicated.
- ... 4 more varying points

Points made by a single run only: 31.

Phrases used by at least half the runs: “0.7 u/h” (19), “early morning” (15).

Evidence cited: 05:00 (24), 0.7u/h (19), 06:00 (15), 99.5mg/dl (10), 97.5% (10), 98.7% (8), 0.8u/h (5), 2.5% (5), 00:00 (4), 98% (3), 4.8% (3), 106mg/dl (3).

decision: Profile / basal 06:00

keep — 26/27 runs

Varying reasoning

- (13/26) Basal 0.8 U/h at 06:00 is appropriate.
- (7/26) No change needed.
- (4/26) Hour 6–7 (06:00–08:00 local) shows 99.5–110.5 mg/dL avg with 98–98.2% TIR.
- (4/26) No change indicated.
- (3/26) Basal 0.8 U/h is appropriate for 06:00–09:59 window.
- (3/26) Basal 06:00–10:00 at 0.8 U/h starts dawn effect management.
- ... 5 more varying points

Points made by a single run only: 50.

Phrases used by at least half the runs: “0.8 u/h” (24), “basal 0.8 u/h” (16).

Evidence cited: 0.8u/h (24), 06:00 (22), 10:00 (12), 98.2% (10), 110.5mg/dl (10), 08:00 (9), 98.7% (7), 132.9mg/dl (7), 99.5mg/dl (6), 07:00 (6), 1.3% (4), 148mg/dl (4).

change — 1/27 runs

Points made by a single run only: 6.

Phrases used by at least half the runs: “change or meal” (1), “hour 10 avg” (1), “00 recommendation add” (1), “00 to support” (1), “10 15 increase” (1), “first measure response” (1), “10 avg 148.0” (1), “rise begins hour” (1), “increase 06 00” (1), “mg/dl peak morning” (1), “glucose accelerates 08” (1), “avg 110.5 mg/dl” (1).

Evidence cited: 08:00 (1), 110.5mg/dl (1), 06:00 (1), 99.5mg/dl (1), 0.7u/h (1), 22mg/dl (1), 11mg/dl (1), 10:00 (1), 15% (1), 0.85u/h (1), 0.92u/h (1), 148.0mg/dl (1).

decision: Profile / basal 10:00

keep — 17/27 runs

Varying reasoning

- (7/17) Observation: Highs are concentrated in hours 13–16 local (13:00–16:00 UTC), where basal 10:00 is still active (0.7 U/h) through 15:59 UTC, then basal 16:00 (0.5 U/h) from 16:00 onward.
- (3/17) Lunch window; glucose avg 148.0 mg/dL, TAR 20.2%.
- (3/17) Hour 10–11 local (08:00–09:00 UTC, hours 9–10) show avg glucose 132.9–148 mg/dL, TIR 78.6–91.1%, with TAR 8.9–20.2%.
- (3/17) Morning rise to 148 mg/dL is likely meal-driven, not basal-insufficient (breakfast meal ~08:00–10:00 local).

- (2/17) Defer basal change until carb logs confirm meal sizes and bolus sizes; if bolus is increased by 20–30% (addressing underbolo), midday basal may be adequate.
- (2/17) Hours 11–12 show elevated glucose (TAR 7.5–14%) consistent with breakfast/lunch overlap and meal bolus inadequacy, not basal issue.
- ... 3 more varying points

Points made by a single run only: 58.

Phrases used by at least half the runs: “0.7 u/h” (16), “basal 0.7” (13), “basal 0.7 u/h” (12), “meal bolus” (11), “hour 10” (10).

Evidence cited: 0.7u/h (16), 10:00 (15), 16:00 (7), 148mg/dl (5), 20.2% (5), 09:00 (4), 148.0mg/dl (4), 0.8u/h (3), 06:00 (2), 163mg/dl (2), 91.1% (2), 08:00 (2).

change — 10/27 runs

Shared reasoning

- (7/10) Basal 10:00–16:00 at 0.7 U/h is insufficient.

Varying reasoning

- (3/10) Afternoon (hours 10–16) is the primary problem: avg 132–163 mg/dL, target 100, TIR 67.9–92.0%, above-range 7.5–32.1%.
- (3/10) If still 130–140 mg/dL, increase to 0.85 U/h.
- (2/10) Midday basal 0.7 U/h produces sustained elevation 141–162 mg/dL (48–62 above target 100) during hours 10–16 (hourly avg 132–163 mg/dL, %AR 7–32%, peak 32% at hour 16).
- (2/10) Implementation: Change basal 10:00 to 0.85 U/h; monitor 2–3 days; if afternoon TIR improves to >75% and TAR <15%, continue.
- (2/10) SMB active but constrained by max_smb_basal_minutes 15 (bolus ceiling 0.175 U/pulse, insufficient to add 0.2–0.3 U/h sustained during meal).
- (2/10) Afternoon basal (10:00–15:59) at 0.7 U/h is insufficient for postprandial carb absorption.
- ... 3 more varying points

Points made by a single run only: 40.

Phrases used by at least half the runs: “0.7 u/h” (8), “basal 10 00” (6), “0.8 u/h” (6), “0.85 u/h” (5), “u/h is insufficient” (5), “163 mg/dl” (5), “target 100” (5), “hours 10” (5), “162 mg/dl” (5).

Evidence cited: 0.7u/h (8), 10:00 (8), 16:00 (7), 0.8u/h (6), 0.85u/h (5), 163mg/dl (5), 162mg/dl (5), 32.1% (4), 32% (3), 14% (3), 50mg/dl (2), 4u/h (2).

decision: Profile / basal 16:00

keep — 21/27 runs

Varying reasoning

- (5/21) Hour 16 avg 162.8 mg/dL (32.1% above range, worst hyperglycemia).
- (3/21) Hour 16 local (15:00–16:00 BST = 14:00–15:00 UTC in UTC hourly_glucose, but 15:00–16:00 UTC in hourly_glucose_local after timezone shift) shows peak afternoon highs (avg 162.8 mg/dL, max 264, TAR 32.1%).
- (3/21) However, the acute 0.5 U/h is likely appropriate for late afternoon (16:00–18:00) when glucose is beginning to decline (hour 17 avg 148.9 mg/dL, hour 18 avg 124.2 mg/dL).
- (3/21) Afternoon/early-evening basal (16:00–18:00) is lowest of day (0.5 U/h).
- (3/21) Basal 0.5 U/h at 16:00 is intentionally low during meal window.
- (2/21) Afternoon basal 0.5 U/h at 16:00 is the lowest point in profile and occurs during worst glucose window (hour 16 avg 162.8 mg/dL, TAR 32.1%).
- ... 5 more varying points

Points made by a single run only: 79.

Phrases used by at least half the runs: “0.5 u/h” (21), “hour 16” (15), “162.8 mg/dl” (14).

Evidence cited: 0.5u/h (21), 16:00 (21), 162.8mg/dl (14), 0.7u/h (9), 32.1% (9), 18:00 (9), 17:00 (5), 10:00 (4), 19:00 (4), 15:00 (3), 14:00 (3), 67.9% (2).

change — 6/27 runs

Varying reasoning

- (3/6) Basal drops from 0.7 U/h (10:00–16:00) to 0.5 U/h (16:00 UTC = 17:00 local) while glucose is still 140–150+ mg/dL and meal absorption is ongoing.
- (3/6) Afternoon basal (16:00–18:00 local, 15:00–17:00 UTC, hour_local 15–16) is the primary deficit.
- (2/6) Raising basal from 0.5 to 0.6 U/h aligns it with surrounding 18:00 segment (0.6 U/h) and provides continuity through the critical post-meal window.
- (2/6) Basal 0.5 U/h is lowest of the day; profile.basal.19_00 (0.7 U/h) shows stable evening.
- (2/6) Secondary recommendation (implement after basal 10:00–16:00 validated in separate 3–5 day trial).

Points made by a single run only: 29.

Phrases used by at least half the runs: “0.5 u/h” (6), “00 local” (4), “0.7 u/h” (4), “afternoon basal” (3), “162.8 mg/dl” (3), “hour 16” (3), “00 utc” (3).

Evidence cited: 0.5u/h (6), 16:00 (6), 18:00 (5), 10:00 (4), 0.7u/h (4), 162.8mg/dl (3), 19:00 (3), 17:00 (3), 0.75u/h (2), 0.65u/h (2), 15:00 (2), 32% (2).

decision: Profile / basal 18:00

keep — 27/27 runs

Shared reasoning

- (20/27) Basal 18:00–19:00 at 0.6 U/h.

Varying reasoning

- (6/27) No change indicated.
- (5/27) Keep at 0.6 U/h.
- (4/27) Hour 18 avg 124.2 mg/dL, hour 19 avg 105.4 mg/dL (lows).
- (4/27) Hour 18 shows avg 124.2 mg/dL (above target 100 but acceptable transition).
- (4/27) Basal 0.6 U/h is appropriate.
- (2/27) Basal increase from 0.5 to 0.6 U/h is modest.
- ... 6 more varying points

Points made by a single run only: 42.

Phrases used by at least half the runs: “0.6 u/h” (25), “hour 18” (19), “124.2 mg/dl” (18), “basal 0.6 u/h” (14).

Evidence cited: 0.6u/h (25), 18:00 (22), 124.2mg/dl (18), 19:00 (18), 80.4% (10), 15.3% (8), 105.4mg/dl (7), 11.9% (7), 16:00 (7), 7.7% (6), 17:00 (6), 0.7u/h (6).

decision: Profile / basal 19:00

keep — 26/27 runs

Varying reasoning

- (9/26) Hour 19–22 local (19:00–22:00: 105–127 mg/dL, 86–92% TIR) is stable, indicating basal 19:00 (0.7 U/h) is appropriate for evening.

- (7/26) Basal 19:00–00:00 at 0.7 U/h.
- (7/26) Basal 0.7 U/h is appropriate for evening.
- (6/26) No change needed.
- (4/26) Hour 19 avg 105.4 mg/dL (15.3% below range, daytime low peak).
- (4/26) Basal 0.7 U/h at 19:00 is an increase from 0.6 U/h at 18:00.
- ... 7 more varying points

Points made by a single run only: 42.

Phrases used by at least half the runs: “0.7 u/h” (24), “basal 0.7 u/h” (18), “hour 19” (14).

Evidence cited: 0.7u/h (24), 19:00 (23), 15.3% (10), 92% (9), 00:00 (8), 23:00 (7), 18:00 (7), 127mg/dl (6), 92.3% (5), 22:00 (5), 105.4mg/dl (5), 120mg/dl (4).

change — 1/27 runs

Points made by a single run only: 6.

Phrases used by at least half the runs: “avg 105.4 mg/dl” (1), “00 with adjacent” (1), “observe next evening” (1), “peaks dependency observe” (1), “hour 19 tbr” (1), “timing/composition unknown zero” (1), “phase after basal/isf” (1), “strategy next phase” (1), “improves but afternoon” (1), “tbr improves” (1), “stabilized is primary” (1), “reduction of 14” (1).

Evidence cited: 0.6u/h (1), 15.3% (1), 19:00 (1), 14% (1), 360min (1), 17:00 (1), 20:00 (1), 18:00 (1), 105.4mg/dl (1).

decision: Profile / CR 00:00

keep — 27/27 runs

Varying reasoning

- (14/27) CR 13 g/U (00:00-04:00) is reasonable.
- (8/27) CR cannot be validated without carb entries.
- (5/27) Keep current value.
- (4/27) CR cannot be validated without carb entries (zero carbs logged).
- (4/27) Keep current values pending meal validation.
- (4/27) Cannot validate without carb logs.
- ... 9 more varying points

Points made by a single run only: 53.

Phrases used by at least half the runs: “13 g/u” (23), “carb logging” (14), “cr 13 g/u” (14).

Evidence cited: 13g/u (23), 00:00 (13), 04:00 (9), 13.2g/u (4), 08:00 (4), 1u (3), 03:59 (3), 50% (1), 99% (1), 0.8u (1), 03:00 (1), 23:00 (1).

decision: Profile / CR 04:00

keep — 27/27 runs

Shared reasoning

- (24/27) CR 11.1 g/U for 04:00–08:00 (early morning, tightest CR value).

Varying reasoning

- (14/27) Cannot validate.
- (9/27) Keep current value pending carb logging.
- (3/27) Keep pending meal validation.
- (2/27) Note: Few meals are eaten at 04:00-08:00 (breakfast is typically 08:00+), so this segment has minimal impact.
- (2/27) No early-morning meals documented (typical breakfast is 08:00).
- (2/27) Structure is reasonable (aggressive CR typical for early morning).
- ... 3 more varying points

Points made by a single run only: 40.

Phrases used by at least half the runs: "11.1 g/u" (24), "early morning" (23), "cr 11.1 g/u" (19), "cannot validate" (14).

Evidence cited: 11.1g/u (24), 04:00 (23), 08:00 (13), 13g/u (6), 07:59 (4), 1u (3), 00:00 (2), 98% (2), 06:00 (2), 106mg/dl (2), 1% (1), 0.8u/h (1).

decision: Profile / CR 08:00

keep — 26/27 runs

Shared reasoning

- (18/26) CR 13.2 g/U (08:00-16:00) covers breakfast and lunch windows.

Varying reasoning

- (12/26) Cannot validate without carb entries.
- (5/26) Keep unchanged until carb logging enables validation.
- (5/26) Keep 13.2 g/U; withhold change pending carb logging.
- (4/26) Keep pending meal logging day.
- (3/26) Confidence in CR error is low without carb logs.
- (3/26) Cannot determine if CR is correct without meal size.
- ... 7 more varying points

Points made by a single run only: 68.

Phrases used by at least half the runs: "13.2 g/u" (24), "cr 13.2 g/u" (18).

Evidence cited: 13.2g/u (24), 08:00 (19), 16:00 (8), 0.2u (5), 148mg/dl (5), 10:00 (4), 10:25 (4), 0.1u (4), 15:59 (4), 1u (3), 0.05u (3), 120min (3).

change — 1/27 runs

Points made by a single run only: 6.

Phrases used by at least half the runs: "hours glp introduced" (1), "16 local 135" (1), "180 while carbs" (1), "130 140 mg/dl" (1), "reactively after glucose" (1), "target 130 140" (1), "mg/dl avg 23" (1), "introduced 2026 08" (1), "insufficient glucose rises" (1), "bolus is proportionally" (1), "basal verified stable" (1), "bolus fades 180" (1).

Evidence cited: 08:00 (1), 162.8mg/dl (1), 32% (1), 235mg/dl (1), 4h (1), 13% (1), 13.2g/u (1), 140mg/dl (1), 16:00 (1), 15% (1), 11.5g/u (1).

decision: Profile / CR 16:00

keep — 26/27 runs

Varying reasoning

- (15/26) CR 10.5 g/U (16:00–20:00) is the tightest segment.
- (13/26) Cannot validate without carb entries.
- (5/26) Keep pending meal validation.
- (4/26) Keep unchanged until carb logging enables validation.
- (4/26) Keep pending carb logging.
- (3/26) Afternoon hyperglycemia (hour 16 TAR 32.1%) is driven by insufficient basal and DIA overestimation, not weak CR.
- ... 7 more varying points

Points made by a single run only: 61.

Phrases used by at least half the runs: “10.5 g/u” (25), “cr 10.5 g/u” (18), “cannot validate” (13).

Evidence cited: 10.5g/u (25), 16:00 (20), 20:00 (9), 162.8mg/dl (5), 19:59 (4), 0.5u/h (3), 32.1% (3), 13.2g/u (3), 04:00 (2), 0.2u (2), 19:00 (2), 15:00 (2).

change — 1/27 runs

Points made by a single run only: 9.

Phrases used by at least half the runs: “context bg rises” (1), “cr is still” (1), “small boluses 0.05” (1), “cr from 10.5” (1), “cr 16 00” (1), “60 min even” (1), “56 50 mg/dl/u” (1), “insufficient against meal” (1), “range worst hyperglycemia” (1), “bolus per meal” (1), “16 avg glucose” (1), “highs persist suggesting” (1).

Evidence cited: 162.8mg/dl (1), 10.5g/u (1), 12:00 (1), 60min (1), 9.5% (1), 16:00 (1), 10% (1), 50mg/dl (1), 0.2u (1), 32.1% (1), 9.5g/u (1), 1u (1).

decision: Profile / CR 20:00

keep — 27/27 runs

Varying reasoning

- (16/27) CR 12.6 g/U for 20:00–00:00 (evening).
- (7/27) Cannot validate without carb entries.
- (6/27) Cannot validate without carb logs.
- (6/27) Keep unchanged until carb logging resumes.
- (6/27) Keep pending meal validation.
- (5/27) Evening CR 12.6 g/U.
- ... 6 more varying points

Points made by a single run only: 43.

Phrases used by at least half the runs: “12.6 g/u” (23), “cr 12.6 g/u” (17).

Evidence cited: 12.6g/u (23), 20:00 (19), 00:00 (10), 19:00 (5), 23:59 (4), 23:00 (3), 0.45u (3), 127mg/dl (2), 22:00 (2), 15.3% (2), 88.6% (2), 117.0mg/dl (2).

decision: Profile / ISF 00:00

keep — 24/27 runs

Varying reasoning

- (5/24) Keep 56 mg/dL/U flat.
- (5/24) Keep flat 56 as baseline.
- (4/24) After carb logging is established and CR is validated, observe correction bolus outcomes (logged carbs + bolus = implied sensitivity) at three time windows (breakfast 08:00–11:00, lunch 12:00–16:00, evening 18:00–23:00) and assess whether segmentation improves TIR.
- (4/24) No time-of-day variation.
- (4/24) Declared ISF 56 mg/dL/U (single flat segment).
- (3/24) However, four dysglycemic swings (2026-08-14 16:20, 2026-08-15 02:30, 2026-08-16 15:45, 2026-08-20 06:54) suggest ISF may interact poorly with DIA 10h and Dynamic ISF (70% range, tightening to 27.8%–42.5% in high-glucose states).
- ... 12 more varying points

Points made by a single run only: 116.

Phrases used by at least half the runs: “56 mg/dl/u” (23), “isf 56” (19), “isf 56 mg/dl/u” (18), “dynamic isf” (17).

Evidence cited: 56mg/dl (23), 70% (15), 0.2u (6), 50mg/dl (4), 10h (3), 160mg/dl (3), 30% (3), 16:20 (3), 24h (3), 16:00 (3), 75mg/dl (2), 50% (2).

change — 3/27 runs

Shared reasoning

- (2/3) Implement immediately (or within 1 day); monitor 3–5 days before basal change.

Points made by a single run only: 22.

Phrases used by at least half the runs: “isf 56” (2), “correction boluses” (2), “profile isf” (2), “response 0.2” (2), “dynamic isf” (2), “56 mg/dl/u” (2), “therapy response windows” (2).

Evidence cited: 70% (2), 56mg/dl (2), 0.2u (2), 258mg/dl (1), 13:05 (1), 60% (1), 130mg/dl (1), 51mg/dl (1), 16:20 (1), 0.05u (1), 80mg/dl (1), 53mg/dl (1).

decision: Profile / target 00:00

keep — 26/27 runs

Varying reasoning

- (9/26) Overnight target 90 mg/dL (00:00–08:00) is sensible for T1D.
- (5/26) No change indicated.
- (4/26) Observed overnight glucose (hours 3–7) averages 99.5–106 mg/dL, slightly above target but within reasonable range.
- (4/26) Overnight target 90 mg/dL.
- (4/26) No change needed.
- (3/26) Target 90 mg/dL at 00:00–07:59 (overnight/early morning).
- ... 7 more varying points

Points made by a single run only: 37.

Phrases used by at least half the runs: “90 mg/dl” (26), “target 90 mg/dl” (25), “overnight target” (20), “overnight target 90” (19).

Evidence cited: 90mg/dl (26), 00:00 (12), 18.5% (8), 08:00 (7), 98.7% (6), 90.6mg/dl (6), 106mg/dl (6), 98% (5), 01:00 (3), 100mg/dl (3), 106.0mg/dl (3), 07:59 (3).

change — 1/27 runs

Points made by a single run only: 6.

Phrases used by at least half the runs: “overnight sensitivity increases” (1), “simplicity during transition” (1), “recommend keeping 100” (1), “low risk buffering” (1), “sensitive and basal” (1), “low risk raising” (1), “tighter daytime control” (1), “tight for sleep” (1), “local 01 00” (1), “implement after isf/basal” (1), “range once isf” (1), “target 90 mg/dl” (1).

Evidence cited: 0.5u/h (1), 08:00 (1), 90.6mg/dl (1), 90mg/dl (1), 00:00 (1), 18.5% (1), 100mg/dl (1), 01:00 (1), 40mg/dl (1).

decision: Profile / target 08:00

keep — 27/27 runs

Varying reasoning

- (14/27) Daytime target 100 mg/dL (08:00–23:00) is standard and appropriate.
- (8/27) Keep target 100 mg/dL.
- (5/27) Afternoon peak (hour 16 avg 162.8 mg/dL) far exceeds target, but this is due to basal and ISF issues, not target setting.
- (5/27) Target 100 mg/dL at 08:00–22:59 (daytime).
- (3/27) Keep target; fix basal.
- (3/27) Do not change target.
- ... 8 more varying points

Points made by a single run only: 56.

Phrases used by at least half the runs: “100 mg/dl” (27), “target 100 mg/dl” (26), “daytime target” (22), “daytime target 100” (20).

Evidence cited: 100mg/dl (27), 08:00 (17), 23:00 (10), 163mg/dl (8), 162.8mg/dl (7), 22:59 (6), 16:00 (5), 10:00 (4), 162mg/dl (4), 32% (3), 98.2% (3), 90mg/dl (3).

decision: Profile / target 23:00

keep — 25/27 runs

Varying reasoning

- (13/25) Late evening target 90 mg/dL (23:00–00:00 local, 22:00–23:00 UTC).
- (10/25) Late evening target transitions back to 90 mg/dL at 23:00.
- (6/25) No change indicated.
- (4/25) Hour 23 shows avg 93.3 mg/dL, TIR 88.6%, achieving target consistently.
- (3/25) No change needed.
- (2/25) Hour 23 shows TIR 88.6%, avg 105.7 mg/dL, TBR 10.1%, indicating moderate control (slightly above target but safe).
- ... 3 more varying points

Points made by a single run only: 18.

Phrases used by at least half the runs: “90 mg/dl” (25), “target 90 mg/dl” (23), “evening target” (14), “late evening” (14), “hour 23” (13).

Evidence cited: 90mg/dl (25), 23:00 (21), 00:00 (11), 105.7mg/dl (11), 93.3mg/dl (6), 23:59 (5), 10.1% (5), 88.6% (5), 88.7% (4), 22:00 (4), 08:00 (2), 00:59 (1).

change — 1/27 runs

Points made by a single run only: 4.

Phrases used by at least half the runs: “target increase hour” (1), “sleep 23 00” (1), “isf and basal” (1), “mg/dl target consistent” (1), “mg/dl during sleep” (1), “basal adjusted” (1), “adjusted for low” (1), “00 boundary sleep” (1), “above current 90” (1), “mg/dl being appropriate” (1), “stabilization user” (1), “22 00 utc” (1).

Evidence cited: 08:00 (1), 90mg/dl (1), 22:00 (1), 23:00 (1), 100mg/dl (1), 105.7mg/dl (1).

decision: Profile / DIA

keep — 24/27 runs

Varying reasoning

- (10/24) DIA 10 hours is standard for rapid-acting insulins (NovoLog, Humalog, Fiasp).
- (4/24) Do not change DIA.
- (4/24) Therapy response windows show 360m glucose changes consistent with 10-hour DIA (insulin is still active at 6 hours, tail continues beyond).
- (4/24) Therapy response windows show multi-hour insulin effects (up to T+360 min), consistent with 10 h DIA.
- (4/24) DIA 10 hours is appropriate for dilute u200 insulin (profile note confirms).
- (4/24) Keep DIA 10h unchanged.
- ... 5 more varying points

Points made by a single run only: 77.

Phrases used by at least half the runs: “dia 10” (22), “10 hours” (22), “dia 10 hours” (19).

Evidence cited: 10h (24), 6h (13), 360min (8), 120min (4), 7h (3), 50% (3), 240min (3), 16:20 (3), 8h (3), 10:25 (2), 0.2u (2), 5h (2).

change — 3/27 runs

Points made by a single run only: 18.

Phrases used by at least half the runs: “dia 10” (3), “high iob” (2), “therapy response” (2).

Evidence cited: 6h (3), 10h (3), 7h (2), 5.5h (2), 50% (1), 21:23 (1), 8h (1), 70% (1), 6u (1), 6.5h (1), 18.5% (1), 19:00 (1).

decision: AAPS / SMB and UAM

keep — 27/27 runs

Varying reasoning

- (15/27) Both SMB and UAM are enabled and functioning correctly.
- (7/27) SMB and UAM are both enabled and functioning appropriately.

- (6/27) Keep both enabled.
- (4/27) SMB fires every 3 min (max 15 min basal cap ~0.15–0.2 U per bolus), UAM detects unannounced meals ($\Delta > 8 \text{ mg/dL/5 min} = \text{min_5m_carbimpact}$).
- (4/27) Both SMB and UAM are appropriately enabled.
- (3/27) After carb logging enabled (recommendation 1), enable `smb_with_cob_enabled=true` (recommendation 2) to allow COB-based SMB.
- ... 10 more varying points

Points made by a single run only: 78.

Phrases used by at least half the runs: “smb and uam” (26).

Evidence cited: 3min (10), 5min (8), 8mg/dl (6), 0.5u (3), 15min (3), 25u (3), 33% (2), 5.935u (2), 0.2u (2), 1.6u (2), 58% (1), 0.05u (1).

decision: AAPS / SMB activation conditions

keep — 24/27 runs

Varying reasoning

- (11/24) SMB activation logic is well-configured. `smb_always_enabled=true` allows SMB to fire when COB=0 (fasting glucose drifts above target), which is appropriate for reactive strategy. `smb_with_cob_enabled=false` prevents SMB when carbs are logged (avoids stacking on COB), which is appropriate design (though moot since user never logs carbs, so COB is always 0). `smb_with_temp_target_enabled=true` allows SMB with user-set low temp targets, appropriate for user’s behavior (representative_events shows multiple temporary target entries, e.g., 2026-08-09 14:41 for 135 mg/dL, 2026-08-15 13:24 for 108 mg/dL). `smb_with_high_temp_target_enabled=false` prevents SMB with high temp targets, appropriate (avoids excessive bolus during deliberate high-target windows).
- (5/24) No change needed.
- (4/24) Keep all settings as-is.
- (3/24) Configuration is appropriate for zero-carb-log workflow.
- (3/24) SMB activation is maximally aggressive (always-on, no carb gate, temp target responsive, high temp targets do not suppress).
- (3/24) SMB activation conditions are appropriate: (1) `smb_always_enabled true` ensures SMB available at all hours (no COB requirement);
- ... 3 more varying points

Points made by a single run only: 44.

Phrases used by at least half the runs: “smb_with_temp_target_enabled true” (19), “smb_with_cob_enabled false” (17), “smb_always_enabled true” (17), “temp target” (14), “true allows” (12).

Evidence cited: 135mg/dl (4), 108mg/dl (2), 140mg/dl (1), 14:41 (1), 13:24 (1), 107mg/dl (1), 12:30 (1), 14:25 (1), 3min (1).

verify — 2/27 runs

Points made by a single run only: 12.

Phrases used by at least half the runs: “carb logging” (2), “smb_with_cob_enabled false” (2), “smb_always_enabled true” (2), “smb_with_temp_target_enabled true” (2), “smb_with_high_temp_target_enabled false” (2), “enabled recommendation smb_with_cob_enabled” (1), “conditions appropriate smb_always_enabled” (1), “conditional change dependent” (1), “now other conditions” (1), “uam delay decision” (1), “detection than uam” (1), “user should enable” (1).

change — 1/27 runs

Points made by a single run only: 6.

Phrases used by at least half the runs: “suppress needed smb” (1), “conditions smb_always_enabled true” (1), “smb to fire” (1), “carbs are absorbed” (1), “board phantom” (1), “even when carbs” (1), “validation carb logged” (1), “isf correction 56” (1), “phantom or real” (1), “carbs on board” (1), “correction 56 90” (1), “absorption window reducing” (1).

Evidence cited: 6h (1), 7h (1), 13:00 (1), 19:00 (1).

decision: AAPS / SMB strength and frequency

keep — 27/27 runs

Varying reasoning

- (4/27) SMB frequency (3 min) and limits (15–20 min basal equiv.) are appropriate for aggressive closed-loop response.
- (4/27) Max UAM SMB 20 min basal is appropriate for unannounced meals.
- (4/27) SMB interval (3 min), max SMB basal (15 min), and UAM max (20 min) are well-configured.
- (3/27) (2) max_smb_basal_minutes=15: SMB bolus ceiling ~0.175 U/pulse at 0.7 U/h basal (15 min of basal).
- (3/27) SMB interval 3 min is aggressive but standard for AAPS+Oref1. max_smb_basal_minutes 15 min (standard ~0.175 U per SMB at typical basals) and max_uam_smb_basal_minutes 20 min are safe and appropriate.
- (3/27) No change needed.
- ... 13 more varying points

Points made by a single run only: 89.

Phrases used by at least half the runs: “20 min” (18), “15 min” (15).

Evidence cited: 3min (21), 20min (17), 15min (12), 0.175u (8), 5min (8), 0.8u/h (7), 0.7u/h (7), 0.2u (4), 1.6u (4), 0.233u (3), 0.3u (3), 0.4u (3).

decision: AAPS / max basal and max IOB limits

keep — 25/27 runs

Shared reasoning

- (15/25) Max basal 12 U/h and max IOB 25 U are well-safe.

Varying reasoning

- (8/25) Observed max temp basal 3.6 U/h is well below 12 U/h.
- (5/25) max_basal_u_per_hour 12 U/h is very high (profile basal is 0.5–0.8 U/h;
- (5/25) No change indicated.
- (4/25) Observed peak suggested basal 4 U/h (33% of max), observed peak IOB 5.9 U (23.7% of max).
- (4/25) (2) max_iob 25 U; peak IOB observed is 5.935 U (24% of limit).
- (4/25) Neither limit is being approached or constraining therapy.
- ... 8 more varying points

Points made by a single run only: 57.

Phrases used by at least half the runs: “12 u/h” (24), “max iob” (17), “basal 12 u/h” (15), “max iob 25” (15), “max basal 12” (15).

Evidence cited: 25u (24), 12u/h (24), 5.935u (16), 4u/h (13), 24% (10), 3.6u/h (8), 5.9u (7), 0.8u/h (6), 20u (4), 33% (3), 19.7u (3), 30% (3).

change — 2/27 runs

Shared reasoning

- (2/2) Max basal 12 U/h: Keep.

Points made by a single run only: 17.

Phrases used by at least half the runs: “meal bolus” (2), “basal 12 u/h” (2), “max iob 25” (2), “max basal 12” (2), “iob 5.935” (2), “first change independent” (1), “boluses 0.2 0.45” (1), “basal ramps hypoglycemia” (1), “overlap high max_iob” (1), “remaining suggests expected” (1), “ceiling for responsive” (1), “concern observed peak” (1).

Evidence cited: 5.935u (2), 12u/h (2), 16:20 (2), 25u (2), 178mg/dl (1), 8u (1), 6u (1), 42mg/dl (1), 4u (1), 0.45u (1), 0.3u (1), 0.2u (1).

decision: AAPS / Autosens and Dynamic ISF

keep — 24/27 runs

Varying reasoning

- (5/24) (3) autosens_raises_target=false; autosens_lowers_target=false: Both false (Autosens does not adjust target).
- (5/24) Autosens disabled (autosens_enabled=false) is appropriate for a stable profile.
- (5/24) For now, keep as-is.
- (4/24) Configuration is appropriate.
- (4/24) Current settings are appropriate.
- (3/24) Autosens disabled;
- ... 8 more varying points

Points made by a single run only: 109.

Phrases used by at least half the runs: “dynamic isf” (21), “autosens disabled” (16).

Evidence cited: 70% (24), 100% (4), 30% (4), 50% (3), 85% (2), 90% (2), 95.2mg/dl (2), 4mg/dl (2), 54mg/dl (1), 39mg/dl (1), 43.2mg/dl (1), 95mg/dl (1).

verify — 2/27 runs

Points made by a single run only: 18.

Phrases used by at least half the runs: “dynamic isf” (2), “autosens is disabled” (2), “glucose swings reason” (1), “glp is ongoing” (1), “consider enabling autosens” (1), “insulin sensitivity autosens” (1), “glp therapy ongoing” (1), “team before changing” (1), “reason for autosens” (1), “keep autosens disabled” (1), “avoid oscillation” (1), “autosens disabled decision” (1).

Evidence cited: 70% (2), 50% (1), 95.2mg/dl (1), 56mg/dl (1).

change — 1/27 runs

Points made by a single run only: 7.

Phrases used by at least half the runs: “iob episodes increasing” (1), “high glucose episodes” (1), “85 reduces aggressiveness” (1), “loop logs adjustment” (1), “increasing adjustment factor” (1), “47.6 mg/dl/u closer” (1), “39.2 mg/dl/u” (1), “highs expected outcome” (1), “risk recommend implementing” (1), “meal bolus change” (1), “allowing less insulin” (1), “combined with reactive” (1).

Evidence cited: 6h (1), 96% (1), 85% (1), 70% (1), 39.2mg/dl (1), 47.6mg/dl (1).

decision: AAPS / carbohydrate absorption model

keep — 22/27 runs

Varying reasoning

- (7/22) min_5m_carbimpact=8 mg/dL/5 min is a moderate UAM threshold.
- (6/22) Min 5m carbimpact (8 mg/dL/5 min = ~96 mg/dL/h) is moderate and appropriate for the system.
- (5/22) min_5m_carbimpact=8 mg/dL/5 min: Standard oref1 threshold.
- (4/22) Meal max absorption 7 h is appropriate for GLP-1-delayed meals (normal 4-5 h, GLP-1 extends to 6-8 h).
- (3/22) No change needed.
- (3/22) Keep both settings.
- ... 6 more varying points

Points made by a single run only: 80.

Phrases used by at least half the runs: “mg/dl/ min” (21), “min_ m_carbimpact” (19), “m_carbimpact mg/dl/ min” (14), “min_ m_carbimpact mg/dl/” (14).

Evidence cited: 5min (22), 8mg/dl (22), 7h (17), 4h (6), 96mg/dl (5), 5h (5), 6h (5), 10h (4), 3h (3), 10:00 (2), 8h (2), 6mg/dl (2).

verify — 3/27 runs

Shared reasoning

- (2/3) Recommendation: keep both values now (lowest-risk approach).

Points made by a single run only: 19.

Phrases used by at least half the runs: “meal_max_absorption_hours is standard” (2), “min_ m_carbimpact” (2), “60 min” (2), “meal logs” (2), “mg/dl/ min” (2), “min post bolus” (2).

Evidence cited: 7h (3), 10h (2), 5min (2), 8mg/dl (2), 60min (1), 120min (1), 90min (1), 6mg/dl (1), 5mg/dl (1), 30% (1).

change — 2/27 runs

Shared reasoning

- (2/2) Current min_5m_carbimpact 8 mg/dL/5 min is above observed slow-meal rise rate.

Points made by a single run only: 11.

Phrases used by at least half the runs: “m_carbimpact mg/dl/ min” (2), “min_ m_carbimpact mg/dl/” (2), “false positive” (2), “basal insufficiency” (2), “min threshold 1.6” (1), “triggers spurious smb” (1), “days for uam” (1), “spurious smb delivery” (1), “min and estimate” (1), “mg/dl/ min observe” (1), “rises reduce spurious” (1), “basal only rises” (1).

Evidence cited: 8mg/dl (2), 7h (2), 5mg/dl (2), 5min (2), 12mg/dl (1), 30mg/dl (1), 1.6mg/dl (1), 3.0mg/dl (1), 2mg/dl (1), 15mg/dl (1), 3mg/dl (1), 2.5h (1).

profile recommendation

basal — 12/27 runs

Shared reasoning

- (8/12) Increase basal rate 10:00–16:00 from 0.7 to 0.8 U/h.

Varying reasoning

- (5/12) If early-morning lows (01:00 local, 18.5% TBR) persist after basal 10:00 fix, reduce basal 00:00 from 0.6 U/h to 0.45–0.55 U/h or apply temporary target 100 mg/dL (22:00–04:00 local) to account for evening bolus IOB tail.
- (3/12) Afternoon glucose (14:00–17:00 local) averages 148–163 mg/dL (target 100 mg/dL) despite system deploying temp basal up to 4 U/h and SMB every 10–15 minutes.
- (3/12) Do NOT change CR, ISF, or DIA further until basal is stable and carb logging is in place; these are interdependent.
- (3/12) Monitor for 2–3 days.
- (2/12) Midday highs (10:00–17:00 avg 132–162 mg/dL, peak 32% above range 16:00) are sustained despite basal 0.7 U/h, indicating basal is insufficient for observed demand.
- (2/12) No other profile parameter is supported for change at this stage (ISF and DIA validation require carb logs; basal 16:00 change depends on separating meal from basal contributions, which requires carb logs).
- ... 2 more varying points

Points made by a single run only: 77.

Phrases used by at least half the runs: “0.7 u/h” (10), “0.5 u/h” (8), “basal 0.7” (8), “basal 0.7 u/h” (7), “increase basal” (6), “0.8 u/h” (6).

Evidence cited: 16:00 (10), 0.7u/h (10), 10:00 (9), 0.5u/h (8), 00:00 (6), 0.8u/h (6), 0.6u/h (5), 32% (5), 17:00 (4), 0.1u/h (4), 162mg/dl (4), 05:00 (3).

ISF — 1/27 runs

Points made by a single run only: 8.

Phrases used by at least half the runs: “130 significantly higher” (1), “undersized by profile” (1), “profile isf 56” (1), “observed correction responses” (1), “correction boluses appear” (1), “isf may need” (1), “days post change” (1), “14 are driven” (1), “overcorrections persist isf” (1), “primary profile change” (1), “mg/dl/u to 90” (1), “change must precede” (1).

Evidence cited: 56mg/dl (1), 90mg/dl (1), 75mg/dl (1), 51mg/dl (1).

basal_cr_isf_dia_validation_sequence — 1/27 runs

Points made by a single run only: 9.

Phrases used by at least half the runs: “dia hours isf” (1), “08 00 segment” (1), “boluses for breakfast” (1), “10 targets 90” (1), “afternoon high evening” (1), “lacks validation” (1), “70 85 mg/dl/u” (1), “reduction if true” (1), “isf dia 10” (1), “reasonable in form” (1), “profile switch aug” (1), “time segmented basal” (1).

Evidence cited: 6h (1), 08:00 (1), 0.80u/h (1), 85mg/dl (1), 100mg/dl (1), 10h (1).

basal, target, CR, ISF, DIA — 1/27 runs

Points made by a single run only: 12.

Phrases used by at least half the runs: “lower 10.5 g/u” (1), “cr validate circadian” (1), “increase despite insulin” (1), “profile error hypoglycemia” (1), “validate basal hour” (1), “dia confirm 10” (1), “0.8 u/h morning” (1), “carb logs 50” (1), “windows show paradoxical” (1), “0.8 u/h cr” (1), “error hypoglycemia clusters” (1), “ramps from 0.6” (1).

Evidence cited: 4.4h (1), 0.8u/h (1), 08:00 (1), 50% (1), 0.6u/h (1), 18.5% (1), 10.5g/u (1), 10h (1), 13.2g/u (1), 11.1g/u (1), 56mg/dl (1), 21:23 (1).

carb logging, then isf verification — 1/27 runs

Points made by a single run only: 11.

Phrases used by at least half the runs: “stabilization before major” (1), “required to confirm” (1), “profile changes carb” (1), “lower risk” (1), “empirical reference delaying” (1), “direction and magnitude” (1), “prebolus immediate bolus” (1), “meal data recalculate” (1), “isf if change” (1), “glucose_change carbs cr” (1), “enable carb logging” (1), “carbs iob subtraction” (1).

Evidence cited: 120mg/dl (1), 56mg/dl (1), 32.1% (1).

carbohydrate_ratio_and_meal_strategy — 1/27 runs

Points made by a single run only: 11.

Phrases used by at least half the runs: “afternoon hyperglycemia simultaneously” (1), “carb reducing post” (1), “basal transition 19” (1), “cr is still” (1), “essential to validate” (1), “patterns are clear” (1), “cr 10.5 9.5” (1), “changes address root” (1), “bolus no prebolus” (1), “coupling via dia” (1), “dia 10 carb” (1), “cr 16 00” (1).

Evidence cited: 162.8mg/dl (1), 10.5g/u (1), 10min (1), 10h (1), 19:00 (1), 16:00 (1), 10% (1), 32.1% (1), 9.5g/u (1).

insulin_duration — 1/27 runs

Points made by a single run only: 12.

Phrases used by at least half the runs: “5.5 testing assess” (1), “still active impossible” (1), “escalated to clinician” (1), “hours evidence therapy” (1), “morning bolus still” (1), “iob overestimate midnight” (1), “hours to 5.5” (1), “different root cause” (1), “bolus recommendation suppressed” (1), “bolus glucose_ 360” (1), “due to perceived” (1), “tbr 18.5 hour” (1).

Evidence cited: 6h (1), 80% (1), 18.5% (1), 5.9u/h (1), 10% (1), 10h (1), 5.5h (1), 14:00 (1).

carbohydrate_logging_and_meal_bolus_execution — 1/27 runs

Points made by a single run only: 14.

Phrases used by at least half the runs: “dia 10 hours” (1), “cr values 10.5” (1), “daytime hyperglycemia hour” (1), “established and meal” (1), “observed small boluses” (1), “carb matched glucose” (1), “small boluses 0.05” (1), “meal sizes is” (1), “boluses 0.05 0.25” (1), “changes are needed” (1), “rise before insulin” (1), “days reassess whether” (1).

Evidence cited: 0.8u/h (1), 08:00 (1), 15.3% (1), 13.2g/u (1), 56mg/dl (1), 15min (1), 67.9% (1), 100mg/dl (1), 0.25u (1), 10h (1), 98.2% (1), 162mg/dl (1).

DIA and basal timing (safety priority), conditional midday basal increase (medium priority, contingent on ISF verification) — 1/27 runs

Points made by a single run only: 9.

Phrases used by at least half the runs: “until isf discrepancy” (1), “utc drop directly” (1), “reducing evening hypoglycemia” (1), “sustained midday hyperglycemia” (1), “mismatch sequential changes” (1), “risk increase basal” (1), “type is confirmed” (1), “00 utc segment” (1), “high iob window” (1), “19 00 utc” (1), “dropping at 16” (1), “hypoglycemia shift basal” (1).

Evidence cited: 6h (1), 0.5u/h (1), 0.8u/h (1), 0.7u/h (1), 10:00 (1), 19:00 (1), 16:00 (1), 18:00 (1), 10h (1), 5.5h (1).

basal_and_targets_and_cr_and_isf_and_dia — 1/27 runs

Points made by a single run only: 15.

Phrases used by at least half the runs: “24 events typical” (1), “actual carbs vs” (1), “overnight control tir” (1), “response adjust cr” (1), “0.7 u/h cannot” (1), “begins logging carbs” (1), “glp therapy cr” (1), “0.8 u/h cr” (1), “profile change” (1), “mg/dl/u flat targets” (1), “isf dia 10” (1), “lows hypoglycemic clusters” (1).

Evidence cited: 0.8u/h (1), 98% (1), 162.8mg/dl (1), 10.5g/u (1), 0.35u (1), 13.2g/u (1), 56mg/dl (1), 0.7u/h (1), 111mg/dl (1), 16:00 (1), 100mg/dl (1), 10% (1).

carb_ratio — 1/27 runs

Points made by a single run only: 8.

Phrases used by at least half the runs: “meal to bolus” (1), “glucose rises category” (1), “ground truth risks” (1), “cr to confirm” (1), “reviewed weeks minimum” (1), “insulin sensitivity rise” (1), “plausible and time” (1), “segments adjust segments” (1), “entries are available” (1), “minimum current segments” (1), “declared cr segments” (1), “change until carb” (1).

Evidence cited: 13.2g/u (1), 12.6g/u (1), 10% (1), 10.5g/u (1).

isf — 1/27 runs

Points made by a single run only: 9.

Phrases used by at least half the runs: “profile isf 56” (1), “lower risk” (1), “basal adjustment” (1), “expected glucose drop” (1), “observed isf 80” (1), “110 mg/dl/u” (1), “isf this change” (1), “adjustment factor concurrently” (1), “isf range 33” (1), “glucose drop dynamic” (1), “risk than basal” (1), “primary profile change” (1).

Evidence cited: 16:00 (1), 70% (1), 110mg/dl (1), 10:00 (1).

meal_strategy_and_carb_logging — 1/27 runs

Points made by a single run only: 17.

Phrases used by at least half the runs: “strategy not profile” (1), “mg/dl daytime daytime” (1), “meal strategy” (1), “high glucose episodes” (1), “70 is aggressively” (1), “glucose response confirming” (1), “deficiency cr inferred” (1), “excess or deficiency” (1), “prioritize carb logging” (1), “40 60 carbs” (1), “responses at 360” (1), “changes lower isf” (1).

Evidence cited: 98% (1), 106mg/dl (1), 2.4u (1), 90mg/dl (1), 23pm (1), 13.2g/u (1), 56mg/dl (1), 70% (1), 105mg/dl (1), 52mg/dl (1), 360min (1), 100mg/dl (1).

basal, cr, isf, dia, target — 1/27 runs

Points made by a single run only: 13.

Phrases used by at least half the runs: “dia 10 hours” (1), “below avg 90.6” (1), “cr isf target” (1), “severe lows 08” (1), “basal or isf” (1), “consistency immediate bolus” (1), “hours show 95” (1), “95 98 tir” (1), “prebolus consistency immediate” (1), “below no lows” (1), “strategy is normalized” (1), “bolus covering 0.05” (1).

Evidence cited: 3% (1), 293mg/dl (1), 98% (1), 0.5u/h (1), 18.5% (1), 10h (1), 13.2g/u (1), 56mg/dl (1), 16:00 (1), 8u (1), 20% (1), 84mg/dl (1).

meal_bolus_timing_and_carbohydrate_entry — 1/27 runs

Points made by a single run only: 13.

Phrases used by at least half the runs: “meal bolus implementation” (1), “isf dia targets” (1), “internally consistent implement” (1), “decrease 13.2 12.5” (1), “lows improve profile” (1), “assess cr consider” (1), “95 98 tir” (1), “consistent implement meal” (1), “carb entries first” (1), “stable if breakfast” (1), “cr may need” (1), “added would indicate” (1).

Evidence cited: 98% (1), 10min (1), 13.2g/u (1), 12.5g/u (1), 56mg/dl (1), 10mg/dl (1), 5mg/dl (1), 10h (1), 20min (1).

Profile defects: flat ISF 56 mg/dL/U (too high, masked by Dynamic ISF compression to 2–4); overnight basal 0.6 U/h (excessive, causing hour 1 low 90.6 mg/dL, 18.5% below range); afternoon CR 13.2 g/U (loosest segment, coinciding with sustained hyperglycemia 130–162 mg/dL, 26% above range). GLP-1 post-2026-08-14 extends meal absorption to 240–360m, but CR and ISF unchanged; reactive bolusing (no prebolus, no carb logging) allows meal boluses to peak at 60m while carbs continue absorbing, causing rebounds to 188–235 mg/dL. AAPS automation (SMB, UAM, temp basal, Dynamic ISF, safety limits) is appropriately configured; no automation defects. Automation is compensating for profile-level errors in real-time. — 1/27 runs

Points made by a single run only: 11.

Phrases used by at least half the runs: “00 to match” (1), “overnight 28 daytime” (1), “risk once isf” (1), “slowed absorption post” (1), “profile isf 56” (1), “u/h 00 00” (1), “reactive bolusing user” (1), “mg/dl/u overnight 00” (1), “hyperglycemia 130 162” (1), “implement three primary” (1), “absorption post 2026” (1), “mg/dl overnight sensitivity” (1).

Evidence cited: 00:00 (1), 16:00 (1), 162mg/dl (1), 0.5u/h (1), 90.6mg/dl (1), 13.2g/u (1), 23:00 (1), 100mg/dl (1), 26% (1), 0.3u (1), 108mg/dl (1), 2.4u (1).

primary recommendation

basal — 9/27 runs

Varying reasoning

- (2/9) Current basal 0.7 U/h in 10:00–16:00 window (and 0.5 U/h in 16:00–18:00) is insufficient.
- (2/9) Midday highs sustained 140+ mg/dL avg (10:00–17:00), peak 32% above range at 16:00 local (17:00 UTC).
- (2/9) Increase midday basal 10:00–17:00 from 0.7 to 0.85 U/h; eliminate 16:00 step-down to 0.5 U/h.
- (2/9) Increase basal rate 10:00–16:00 window from 0.7 to 0.8 U/h to address primary afternoon hyperglycemia driver.

Points made by a single run only: 55.

Phrases used by at least half the runs: “0.5 u/h” (8), “0.7 u/h” (7).

Evidence cited: 0.5u/h (8), 0.7u/h (7), 16:00 (6), 10:00 (6), 17:00 (5), 32% (5), 0.1u/h (3), 0.8u/h (3), 00:00 (2), 0.6u/h (2), 162.8mg/dl (2), 14:00 (2).

meal_strategy — 4/27 runs

Varying reasoning

- (2/4) Implement carb logging for 3–5 days to validate profile and meal patterns.

Points made by a single run only: 22.

Phrases used by at least half the runs: “carb logging” (3), “cr validation” (2), “implement carb logging” (2).

Evidence cited: 80% (1), 4h (1), 15min (1), 5.5u (1), 2u (1), 7u (1), 30% (1).

ISF — 1/27 runs

Points made by a single run only: 5.

Phrases used by at least half the runs: “response 80 150” (1), “corrections appear undersized” (1), “undersized by profile” (1), “mg/dl/u therapy_response_windows median” (1), “driven partly” (1), “profile isf 56” (1), “mechanism of hypoglycemia” (1), “excessive actual insulin” (1), “14 are driven” (1), “baseline correction strength” (1), “100 will establish” (1), “56 to 90” (1).

Evidence cited: 60% (1), 90mg/dl (1), 56mg/dl (1), 51mg/dl (1).

Data and meal strategy — 1/27 runs

Points made by a single run only: 4.

Phrases used by at least half the runs: “days prevents cr” (1), “correction dia validation” (1), “carb logging timestamps” (1), “measure bolus decay” (1), “prevents cr validation” (1), “restart carb logging” (1), “carb logging immediately” (1), “pattern and validate” (1), “strategy refinement carb” (1), “logging breakfast lunch” (1), “zero carbs logged” (1), “14 days prevents” (1).

safety — 1/27 runs

Points made by a single run only: 8.

Phrases used by at least half the runs: “boluses 0.2 0.45” (1), “maximum when two” (1), “insulin suggests expected” (1), “basal ramps hypoglycemia” (1), “iob 5.935 2026” (1), “profile changes” (1), “meal bolus iob” (1), “25 to immediately” (1), “peak iob 5.935” (1), “first action independent” (1), “ramps hypoglycemia clusters” (1), “nadir 41 42” (1).

Evidence cited: 0.3u (1), 5.935u (1), 8u (1), 6u (1), 42mg/dl (1), 4u (1), 0.45u (1), 25u (1), 21:09 (1).

meal strategy — 1/27 runs

Points made by a single run only: 12.

Phrases used by at least half the runs: “meal timing clarity” (1), “proceed to isf” (1), “days review therapy” (1), “alertness vs uam” (1), “app after days” (1), “logged 14 day” (1), “timing clarity carb” (1), “highest information change” (1), “time vs bolus” (1), “prevents cr validation” (1), “bolus time gap” (1), “enter in app” (1).

Evidence cited: 2min (1).

Carbohydrate logging (highest-leverage behavior change) — 1/27 runs

Points made by a single run only: 5.

Phrases used by at least half the runs: “cannot be verified” (1), “glucose peak 20” (1), “boluses forcing reliance” (1), “days/week would allow” (1), “activity in afternoon” (1), “carbs to account” (1), “two dysglycemic reversals” (1), “50 less smb” (1), “rise as meals” (1), “allow meal boluses” (1), “reduce afternoon glucose” (1), “uam detection carbs” (1).

Evidence cited: 13.2g/u (1), 30mg/dl (1), 50% (1).

meal_logging — 1/27 runs

Points made by a single run only: 5.

Phrases used by at least half the runs: “nocturnal lows 11” (1), “expected outcome coefficient” (1), “minimum consecutive days” (1), “shape and guide” (1), “see if meals” (1), “peaks carb duration” (1), “variation 32.6 25” (1), “entry logging user” (1), “one meal” (1), “near glp doses” (1), “hyperglycemia 32 15” (1), “meal size inferred” (1).

Evidence cited: 32% (1), 12:00 (1), 18% (1), 5% (1), 20% (1), 32.6% (1), 28% (1).

dia — 1/27 runs

Points made by a single run only: 10.

Phrases used by at least half the runs: “carryover into midnight” (1), “aggressively at lunch” (1), “tbr 18.5 excessive” (1), “driven carryover hypothesis” (1), “iob at 14” (1), “driver of midnight” (1), “faster reducing midnight” (1), “clear iob 80” (1), “bolus glucose_ 360” (1), “driver of afternoon” (1), “largely cleared loop” (1), “aggressive bolus strategy” (1).

Evidence cited: 6h (1), 80% (1), 18.5% (1), 4h (1), 60% (1), 360min (1), 10h (1), 5.5h (1), 14:00 (1).

behavioral_and_logging — 1/27 runs

Points made by a single run only: 5.

Phrases used by at least half the runs: “days prevent direct” (1), “timing and carb” (1), “spikes carb logging” (1), “day lunch expand” (1), “unable to prevent” (1), “initiate carbohydrate logging” (1), “meal bolus timing” (1), “zero carb entries” (1), “validation of cr” (1), “initial hyperglycemia spikes” (1), “start with one” (1), “entries across 14” (1).

Data Integrity – ISF Verification — 1/27 runs

Points made by a single run only: 6.

Phrases used by at least half the runs: “loop over ride” (1), “acting humalog/novolog/fiasp vs” (1), “causes json export” (1), “profile changes” (1), “vs intermediate levemir/nph” (1), “blocks all derivative” (1), “loop calculated isf” (1), “ride concurrently insulin” (1), “mg/dl/u this 10” (1), “cr assessments possible” (1), “verify pump resident” (1), “clarifications all profile” (1).

Evidence cited: 4mg/dl (1), 56mg/dl (1), 10h (1).

carb_logging — 1/27 runs

Points made by a single run only: 3.

Phrases used by at least half the runs: “uam accuracy validate” (1), “timing and meal” (1), “validate cr time” (1), “implement carb entry” (1), “carb amounts timing” (1), “different times” (1), “whether afternoon lows” (1), “start recording meal” (1), “required to ground” (1), “determine whether afternoon” (1), “recording meal carb” (1), “carb entry logging” (1).

isf — 1/27 runs

Points made by a single run only: 9.

Phrases used by at least half the runs: “swing is 33” (1), “observed sensitivity” (1), “increase both baseline” (1), “maximum isf swing” (1), “isf 56” (1), “change aaps smb/uam” (1), “isf variance evident” (1), “sensitivity this change” (1), “proportional to profile” (1), “factor cannot close” (1), “windows show actual” (1), “evident implement first” (1).

Evidence cited: 70% (1), 204mg/dl (1), 110mg/dl (1).

meal_bolus_strategy — 1/27 runs

Points made by a single run only: 6.

Phrases used by at least half the runs: “afternoon 162 mg/dl” (1), “89 91 implement” (1), “carbs are absorbed” (1), “heights by 10” (1), “creating peaks breakfast” (1), “peaks breakfast 148” (1), “148 mg/dl 20.2” (1), “lag and afternoon” (1), “breakfast 148 mg/dl” (1), “uam reactive uam” (1), “uam has 10” (1), “add modest prebolus” (1).

Evidence cited: 91% (1), 86% (1), 148mg/dl (1), 20.2% (1), 2u (1), 1u (1), 32.1% (1), 20mg/dl (1), 162mg/dl (1), 20min (1).

meal_carbs — 1/27 runs

Points made by a single run only: 5.

Phrases used by at least half the runs: “tuning cr isf” (1), “patterns meal logs” (1), “logged over 14” (1), “automation all profile” (1), “carb ratios bolus” (1), “validation of carb” (1), “logs will clarify” (1), “carb entries logged” (1), “carbohydrate logging zero” (1), “suggesting appetite suppression” (1), “meal specific automation” (1), “basal adjustment carbs” (1).

profile — 1/27 runs

Points made by a single run only: 6.

Phrases used by at least half the runs: “time overnight 00” (1), “isf 70 compression” (1), “profile isf 56” (1), “need for compression” (1), “reduces dynamic isf” (1), “effectiveness and overnight” (1), “sensitive matches observed” (1), “00 28 mg/dl/u” (1), “lower from 70” (1), “small bolus effectiveness” (1), “mg/dl/u matches observed” (1), “isf revision reassess” (1).

Evidence cited: 08:00 (1), 00:00 (1), 70% (1), 28mg/dl (1), 56mg/dl (1), 23:00 (1), 4mg/dl (1), 40mg/dl (1).

summary

27 runs

Shared reasoning

- (18/27) 14-day CGM and insulin delivery telemetry (2026-08-07 to 2026-08-21, 98.7% coverage) shows good overall control (86% time in range) but significant variability (CV 32.6%, SD 39.7 mg/dL) driven by bimodal glucose patterns: sustained midday highs (10:00–17:00 avg 132–162 mg/dL, peak 32% above range at 16:00) alternating with afternoon and early-night lows (16:00–18:00 and 23:00–03:00, 5–18% below range, min 41 mg/dL).

Varying reasoning

- (6/27) Daytime hyperglycemia (32% TAR afternoon hour 16:00 UTC) correlates with insufficient basal (0.5 U/h at 16:00, 0.7 U/h at 10:00–13:00).
- (6/27) Data quality excellent (98.7% CGM coverage).
- (4/27) Patient with type 1 diabetes is using AAPS closed-loop with zero carbohydrate logging over a 14-day period (2026-08-07 to 2026-08-21).
- (4/27) Profile settings (basal, CR, ISF, DIA) are plausibly correct but cannot be validated without carb data.
- (4/27) Automation (SMB, UAM, Dynamic ISF) is functioning correctly but cannot compensate for profile-level basal defect.
- (4/27) Zero meal carbs logged; system relies entirely on basal, SMB/UAM, and unvalidated CR/ISF.
- ... 18 more varying points

Points made by a single run only: 147.

Phrases used by at least half the runs: “closed loop” (17), “meal strategy” (16), “dynamic isf” (16), “carb logging” (15), “14 day” (15), “meal bolus” (14), “bolus strategy” (14), “56 mg/dl/u” (14).

Evidence cited: 86% (21), 98.7% (16), 56mg/dl (14), 17:00 (14), 16:00 (14), 10:00 (13), 10h (11), 32% (10), 32.6% (8), 19:00 (7), 5.1% (7), 70% (7).

issues

27 runs

Varying reasoning

- (11/27) At 16:00 UTC, midday meal carbs are still being absorbed (glucose has not yet peaked; peak is 15:00–16:00 local = 16:00–17:00 UTC), and SMB/correction boluses from 10:00–15:00 UTC still have 6–9 h IOB remaining (DIA = 10 h).
- (7/27) Recurrent critical lows (5 readings <50 mg/dL on 2026-08-13, 2026-08-14, 2026-08-21).
- (7/27) Root cause is zero carb logging; without meal context, system cannot distinguish meal-caused glucose rise from basal-caused rise, leading to reactive bolusing. } {‘issue’: ‘Hypoglycemia clusters on 2026-08-13 to 2026-08-14 (nadir 41-42 mg/dL, two separate clusters, 4 + 3 extremes <55)’, ‘severity’: ‘safety’, ‘evidence’: “glucose_extremes: 2026-08-13 17:00-17:14 UTC (four readings: 51, 47, 42, 45 mg/dL, nadir 42), 2026-08-14 18:04-18:14 UTC (three readings: 48, 41, 41 mg/dL, nadir 41).
- (6/27) No meal bolus strategy declared or visible in aaps_profile.analysis_context (empty meal_strategies array).
- (5/27) {‘area’: ‘Carb logging absent’, ‘description’: ‘Zero carb entries over 14 days.
- (4/27) But prior 48 hours saw three device changes: site change 08-13 21:45, profile switch 08-13 21:23 (new ‘u200 dilute’ profile), sensor change 08-11.
- ... 43 more varying points

Points made by a single run only: 422.

Phrases used by at least half the runs: “dynamic isf” (20), “isf 56” (19), “meal bolus” (19), “carb entries” (19), “zero carb” (18), “56 mg/dl/u” (18), “zero carb entries” (16), “14 days” (16), “meal strategy” (15), “isf 56 mg/dl/u” (15), “0.7 u/h” (15), “meal absorption” (15).

Evidence cited: 56mg/dl (20), 16:00 (19), 17:00 (16), 0.2u (15), 0.7u/h (15), 10:00 (15), 70% (14), 32.1% (14), 18:00 (13), 6h (12), 4h (12), 19:00 (11).

step findings: data_quality

27 runs

Shared reasoning

- (25/27) CGM coverage 98.7% over 14 days (3,995 valid entries, 10 gaps, largest 65 min).

Varying reasoning

- (16/27) Profile is complete (7 basal segments, 5 CR segments, 1 ISF segment, 3 target segments, DIA 10h).
- (12/27) Treatment telemetry complete: 2315 total treatments, 655 insulin entries, zero carb entries, zero extended carb entries.
- (12/27) Telemetry includes 2,315 treatments (655 insulin entries, 1,660 temp basals), 6,214 device statuses, and 27 events.
- (8/27) Zero invalid or duplicate entries.
- (7/27) AAPS profile active throughout period per profile_context.historical_applicability=yes, updated 2026-08-21 21:08:10 UTC.
- (7/27) 10 gaps, largest 65 minutes.
- ... 15 more varying points

Points made by a single run only: 39.

Phrases used by at least half the runs: “cgm coverage” (22), “valid entries” (18), “14 days” (17), “10 gaps” (17), “655 insulin” (16), “dia 10” (16), “cgm coverage 98.7” (16), “cr segments” (15), “65 min” (15), “basal segments” (14), “invalid entries” (14), “aaps profile” (14).

Evidence cited: 65min (27), 98.7% (27), 10h (16), 5min (5), 24h (3), 21:15 (2), 21:09 (2), 56mg/dl (2), 21:08 (2), 5.0min (1), 21:23 (1), 21:45 (1).

step findings: safety_overview

27 runs

Shared reasoning

- (17/27) Clusters: 2026-08-13 17:04–17:14 (4 readings 42–47 mg/dL), 2026-08-14 18:04–18:14 (3 readings 41–48 mg/dL), 2026-08-21 06:19 (1 reading 42 mg/dL).

Varying reasoning

- (14/27) Clusters: Aug 13 17:00–17:15 (51, 47, 42, 45 mg/dL), Aug 14 18:04–18:14 (48, 41, 41 mg/dL), Aug 21 06:19 (42 mg/dL).
- (13/27) Hypoglycemia: 5.1% time below 70 mg/dL overall.
- (11/27) Variability high: SD 39.7 mg/dL, CV 32.6%.
- (10/27) Coefficient of variation 32.6%; standard deviation 39.7 mg/dL.
- (9/27) Afternoon window 10:00–17:00 local shows sustained elevated glucose (averages 132–162 mg/dL) with TIR only 67.9–92.0%, worst at 16:00 local (162.8 mg/dL avg, 67.9% TIR, 32.1% above range).
- (8/27) Time below range by hour: hour 0 (midnight, 18.5% TBR), hour 1 (01:00, 18.5% TBR), hour 19 (18:00 UTC / 19:00 local, 15.3% TBR).
- ... 22 more varying points

Points made by a single run only: 78.

Phrases used by at least half the runs: “42 mg/dl” (19), “time below” (19), “above range” (19), “time above” (18), “below range” (17), “293 mg/dl” (15), “time below range” (15), “cv 32.6” (14), “time above range” (14), “51 mg/dl” (14).

Evidence cited: 5.1% (27), 32.6% (26), 8.9% (25), 42mg/dl (19), 17:00 (17), 293mg/dl (15), 32.1% (15), 18:14 (14), 51mg/dl (14), 06:19 (13), 18:04 (12), 70mg/dl (12).

step findings: basal_targets

27 runs

Shared reasoning

- (20/27) Basal segments: 0.6 U/h (00:00-05:00), 0.7 U/h (05:00-06:00), 0.8 U/h (06:00-10:00), 0.7 U/h (10:00-16:00), 0.5 U/h (16:00-18:00), 0.6 U/h (18:00-19:00), 0.7 U/h (19:00-00:00).

Varying reasoning

- (14/27) Overnight basal (00:00–07:00 local): 0.6–0.8 U/h structure adequate.
- (8/27) Afternoon basal (0.7 U/h at 10:00, 0.5 U/h at 16:00) is problematic.
- (5/27) TAR 7.5–32.1% (worst hour 16: 162.8 mg/dL avg, 32.1% TAR, 67.9% TIR).
- (5/27) Early morning (06:00–10:00) shows gentle rise consistent with dawn effect; basal 0.8 U/h at 06:00 appears appropriate.
- (4/27) Morning (08:00–10:00): Glucose rises 110.5 → 132.9 → 148.0 mg/dL; basal is stable 0.8 U/h (hours 8–9) then drops to 0.7 U/h (hour 10).
- (4/27) Hours 04–07 excellent (TIR 95–98.7%, TBR 1.3–4.8%).
- ... 20 more varying points

Points made by a single run only: 147.

Phrases used by at least half the runs: “0.7 u/h” (25), “0.8 u/h” (25), “0.5 u/h” (23), “0.6 u/h” (20), “basal 0.6” (16), “162.8 mg/dl” (16), “00 local” (15), “target 90” (14), “basal 0.7” (14).

Evidence cited: 00:00 (26), 0.8u/h (25), 0.7u/h (25), 16:00 (24), 0.5u/h (23), 10:00 (22), 08:00 (21), 0.6u/h (20), 17:00 (19), 18:00 (19), 18.5% (18), 06:00 (17).

step findings: profile_cr_isf_dia

27 runs

Shared reasoning

- (24/27) CR: 13.2 g/U (08:00–16:00 breakfast/lunch), 10.5 g/U (16:00–20:00), 12.6 g/U (20:00–00:00), 13 g/U (00:00–04:00), 11.1 g/U (04:00–08:00).

Varying reasoning

- (12/27) ISF: flat 56 mg/dL/U.
- (9/27) ISF single flat segment 56 mg/dL/U (00:00–23:59).
- (9/27) Cannot validate CR without carb entries.
- (6/27) Dynamic ISF active (70% adjustment factor) reduces effective ISF to 39–56 mg/dL/U during high-IOB periods; system is bottlenecked even with real-time adjustment.
- (6/27) Dynamic ISF (enabled, 70% adjustment factor) is mostly inactive (sensitivity_ratio = 1.0 in 85% of loops), suggesting base ISF 56 is approximately correct.
- (6/27) DIA: 10 hours (unusually long for rapid-acting insulin; standard is 4–6 hours).
- ... 19 more varying points

Points made by a single run only: 191.

Phrases used by at least half the runs: “56 mg/dl/u” (27), “dia 10” (24), “13.2 g/u” (23), “isf 56” (21), “10 hours” (21), “dynamic isf” (21), “12.6 g/u” (20), “10.5 g/u” (20), “11.1 g/u” (17), “therapy response” (16), “dia 10 hours” (14).

Evidence cited: 10h (27), 56mg/dl (27), 16:00 (23), 13.2g/u (23), 08:00 (20), 10.5g/u (20), 12.6g/u (20), 04:00 (19), 20:00 (18), 00:00 (17), 11.1g/u (17), 70% (15).

step findings: automation_smb_uam

27 runs

Shared reasoning

- (22/27) SMB activation: smb_always_enabled (true), smb_with_cob_enabled (false), smb_after_carbs_enabled (false), smb_with_temp_target_enabled (true), smb_with_high_temp_target_enabled (false).
- (17/27) SMB and UAM: Both enabled (true).

Varying reasoning

- (15/27) Max SMB basal 15 min (~0.175 U per SMB at 0.7 U/h basal) and max UAM SMB 20 min (~0.23 U) are conservative limits.
- (11/27) Carb absorption model: min_5m_carbimpact=8 mg/dL/5 min (UAM threshold), meal_max_absorption_hours=7.
- (10/27) Max basal 12 U/h, max IOB 25 U.
- (7/27) Peak IOB observed 5.935 U (2026-08-21 21:09); peak temp basal enacted 3.6 U/h (2026-08-14 18:40).
- (7/27) SMB strength and frequency: interval 3 min, max_smb_basal_minutes 15, max_uam_smb_basal_minutes 20.
- (5/27) Max IOB: 25 U (observed max 5.935 U, 24% usage).
- ... 21 more varying points

Points made by a single run only: 170.

Phrases used by at least half the runs: “smb_always_enabled true” (21), “smb_with_temp_target_enabled true” (20), “mg/dl/ min” (20), “smb_with_cob_enabled false” (19), “max_smb_basal_minutes 15” (19), “20 min” (15), “max_uam_smb_basal_minutes 20” (15), “m_carbimpact mg/dl/ min” (15), “min_ m_carbimpact mg/dl/” (15), “12 u/h” (14), “true smb” (14), “zero carb” (14).

Evidence cited: 5min (20), 8mg/dl (20), 3min (18), 25u (16), 5.935u (14), 12u/h (14), 20min (14), 0.7u/h (12), 15min (12), 4u/h (11), 0.2u (10), 1.6u (9).

step findings: automation_sensitivity_limits

27 runs

Varying reasoning

- (16/27) Dynamic ISF enabled (dynamic_sensitivity_enabled true) at 70% adjustment factor (dynamic_isf_adjustment_factor_percent 70).
- (14/27) Max basal 12 U/h, max IOB 25 U both appropriate for closed-loop.
- (14/27) Autosens is disabled (autosens_enabled = false).
- (11/27) Autosens_raises_target false, autosens_lowers_target false.
- (8/27) Carb absorption model: min_5m_carbimpact=8 mg/dL/5 min, meal_max_absorption_hours=7h (standard).
- (6/27) Safety multipliers: max_daily_safety_multiplier=5.0, current_basal_safety_multiplier=6.0.
- ... 22 more varying points

Points made by a single run only: 195.

Phrases used by at least half the runs: “dynamic isf” (26), “autosens_enabled false” (20), “autosens disabled” (20), “dynamic_sensitivity_enabled true” (19), “adjustment factor” (19), “disabled autosens_enabled false” (17), “enabled dynamic_sensitivity_enabled true” (17), “12 u/h” (17), “isf 56” (15), “dynamic isf enabled” (15), “70 adjustment” (15).

Evidence cited: 70% (26), 12u/h (17), 25u (16), 4u/h (11), 5min (8), 5.935u (8), 56mg/dl (8), 8mg/dl (7), 7h (6), 30% (6), 100mg/dl (6), 5.9u (5).

step findings: meal_bolus_strategy

26 runs

Shared reasoning

- (21/26) No declared meal strategy (aaps_profile.analysis_context.meal_strategies empty array).

Varying reasoning

- (13/26) Meal timing inferred from hourly glucose pattern: breakfast ~08:00 (peak 09:00–10:00 at 132.9–148 mg/dL), lunch ~12:00–13:00 (peak 141.3–135 mg/dL), afternoon snack/meal ~14:00–16:00 (sustained peak 148–163 mg/dL, TAR 23–32%), dinner ~17:00–20:00 (rise then fall 17:00–19:00 due to high basal, rise 20:00–21:00), evening snack possible ~22:00.
- (6/26) Examples: 2026-08-14 16:20 (0.2 U from 234 mg/dL), 2026-08-15 11:25 (2.4 U from 181 mg/dL), 2026-08-19 15:55 (0.65 U from 264 mg/dL).
- (6/26) E-carbs: not used (zero extended carb entries).
- (5/26) Observed strategy (telemetry inference, CHECKPOINT 3): zero meal boluses logged; all 24 therapy_response_windows labeled 'Correction Bolus'.
- (4/26) Observed strategy: reactive, SMB/UAM only.
- (3/26) Examples: 2026-08-14 10:25 bolus 0.2 U at glucose 100 mg/dL start, glucose rose +77 mg/dL by 120m (meal not prebolused, bolus given after meal absorption started).
- ... 21 more varying points

Points made by a single run only: 213.

Phrases used by at least half the runs: "aaps_profile analysis_context" (19), "analysis_context meal_strategies" (17), "immediate bolus" (16), "observed strategy" (15), "aaps_profile analysis_context meal_strategies" (15), "meal strategy" (14), "glucose rise" (13), "meal bolus" (13).

Evidence cited: 0.2u (19), 2.4u (17), 11:25 (12), 4h (11), 10:25 (11), 08:00 (11), 3min (10), 12:00 (10), 13.2g/u (8), 19:00 (8), 30min (7), 100% (7).

step findings: meal_ecarbs_absorption

26 runs

Shared reasoning

- (24/26) Zero e-carb entries (extended_carb_entries = 0, average_extended_carb_duration_min = null).

Varying reasoning

- (5/26) Example: Aug 15 11:25 bolus 2.4 U at BG 181 shows drop 181→94 in 120 min (no buffering from e-carbs) then rebound 94→130 by 180m and 188 by 240m (meal carbs or next meal resuming absorption).
- (5/26) Carb absorption model configured: min_5m_carbimpact=8 mg/dL/5 min, meal_max_absorption_hours=7 h.
- (4/26) Observed glucose absorption patterns: breakfast peak 08:00–11:00 (2–3 h post-meal, typical); lunch peak 14:00–16:00 (3–4 h post-meal, delayed); dinner peak 20:00–22:00 (2–3 h post-meal, typical).
- (4/26) User is on GLP-1 therapy (three documented injections: 2026-08-14 08:14, 2026-08-17 12:03, 2026-08-20 21:34).
- (4/26) Examples: 2026-08-13 13:05 bolus 0.2U→-26 at 120m, then +32 at 360m (meal carbs still active at 360m);
- (4/26) GLP-1 mechanism delays gastric emptying, extending meal absorption from normal 2–3 hours to 4–6 hours.
- ... 13 more varying points

Points made by a single run only: 156.

Phrases used by at least half the runs: "carb entries" (21), "meal absorption" (20), "extended_carb_entries average_extended_carb_duration_min null" (18), "extended carb" (13).

Evidence cited: 4h (15), 3h (14), 7h (12), 6h (10), 5h (9), 10h (9), 11:25 (9), 2h (8), 5min (8), 120min (7), 8mg/dl (7), 240min (6).

step findings: synthesis_plan

27 runs

Varying reasoning

- (8/27) Basal 00:00 appears appropriate for fasting (hour 6–8 post-midnight are excellent, 98% TIR), but at midnight (hour 0, 23:00 local) TBR is 11.4%, and hour 1 (01:00 local, 00:00 UTC on basal 00:00) is 18.5% TBR.
- (5/27) Dynamic ISF (70%) is active compensation; no ISF change without disabling Dynamic ISF first.
- (4/27) PRIMARY: Basal insufficiency in 10:00–16:00 afternoon window.
- (4/27) CR 10.5–13.2 g/U reasonable but unvalidated without meal logs.
- (4/27) Contradiction audit: All CHECKPOINT 1–3 claims validated against aaps_profile, profile_context, decision_inventory, and telemetry.
- (3/27) Secondary recommendation: increase basal 10:00–16:00 from 0.7 to 0.85 U/h (13% increase, conservative); reassess 16:00 reduction afterward.
- ... 31 more varying points

Points made by a single run only: 482.

Phrases used by at least half the runs: “meal bolus” (22), “dynamic isf” (20), “carb logging” (19), “meal strategy” (17), “zero carb” (16), “isf 56” (16), “0.7 u/h” (15).

Evidence cited: 16:00 (18), 70% (15), 0.7u/h (15), 10h (13), 10:00 (13), 0.6u/h (12), 0.2u (10), 0.5u/h (10), 56mg/dl (9), 17:00 (9), 19:00 (8), 4h (8).

meal strategy

27 runs

Varying reasoning

- (4/27) No e-carbs are used.
- (3/27) Telemetry shows boluses are reactive (given after glucose peak or during rise, e.g., 2026-08-14 10:25 BG 100 during upturn, 2026-08-15 11:25 BG 181 at peak), not proactive (not before meal).
- (3/27) Prebolus is 0–10 minutes (bolus given at meal time or just before).
- (2/27) Implement carb logging.
- (2/27) SMB every 3 min + UAM 8 mg/dL/5 min threshold detect carbs only after they enter bloodstream (30–60 min post-meal, 60–90 min post-meal for GLP-1), allowing initial high rise (50–88 mg/dL within 60 min) that is not prevented.
- (2/27) Afternoon BG peak 162.8 mg/dL (hour 16) occurs 4 hours after lunch meal start, not immediately after bolus, indicating meal carbs are being absorbed over extended period and basal 0.7 U/h is insufficient to offset demand.
- ... 8 more varying points

Points made by a single run only: 396.

Phrases used by at least half the runs: “carb logging” (21), “meal bolus” (18), “post meal” (17), “immediate bolus” (15), “bolus strategy” (14), “15 min” (14).

Evidence cited: 4h (15), 15min (14), 0.2u (13), 5min (13), 3h (12), 3min (11), 30min (9), 100% (9), 10min (9), 13.2g/u (9), 60min (8), 50% (8).